

Chapter 10 (New): The Clinical Pillar — Redesigning Care Delivery for a Transformed System

The Blueprint for Health, Behavioral Health Integration, CCBHC Expansion, Primary Care Transformation, and Vermont's Clinical Quality Architecture

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program; AHS Mental Health Integration Pilot Evaluation 2025; Oliver Wyman Act 167 Recommendations; AHS Transformation Reports (2025); Act 68 of 2025; Vermont RHT Program Application; SAMHSA CoCM resources; Population Health Management journal (Blueprint ROI study).

The Clinical Pillar: What It Is and Why It Is Not the Same as Clinical Quality

A common confusion in healthcare transformation discussions conflates the clinical pillar with clinical quality measurement — HEDIS scores, Star ratings, readmission rates. Clinical quality measurement is important, and this chapter addresses it, but the clinical pillar is a larger concept. It encompasses the design of care delivery itself: how care is organized, who provides it, where it is delivered, and how the various elements of the healthcare system are coordinated around the patient's actual health needs rather than around administrative or financial convenience.

In the context of Vermont's transformation, the clinical pillar question is specific and urgent: what does primary care look like in a system no longer organized around hospitals as the default site of care? What does behavioral health integration look like in a state with 245 ED visits per 10,000 for suicide ideation and self-harm, and inadequate community mental health capacity? What does care coordination look like for Vermont's rapidly growing elderly population, with the highest-cost, most complex needs? And how do these clinical design questions connect to the payment reform and operational transformation happening simultaneously?

Vermont has more developed answers to these clinical design questions than most states, because Vermont has been running the Blueprint for Health — the most sophisticated all-payer primary care transformation program in the country — for more than fifteen years. The Blueprint is not a pilot or a demonstration. It is operating infrastructure, funded by all payers, embedded in primary care practices across all 14 Hospital Service Areas, and producing measurable results. Understanding the Blueprint is the foundation for understanding Vermont's clinical pillar strategy.

Section 1: The Vermont Blueprint for Health — Fifteen Years of Primary Care Transformation

The Blueprint for Health is Vermont's all-payer primary care transformation program, established in 2006 and reaching statewide implementation by 2013. It is, in the view of most independent analysts, the most successful sustained primary care transformation initiative in American health policy. Its longevity, its all-payer architecture, its integration with Vermont's health data infrastructure, and its documented results make it the clinical foundation on which Vermont's broader healthcare transformation depends.

The Blueprint Architecture

The Blueprint operates through three integrated components that together constitute a population health management system at the primary care level:

Component	What it is	How it is funded	What it does
Patient-Centered Medical Home (PCMH)	Primary care practices recognized by NCQA as meeting advanced standards for care coordination, quality improvement, population management, and patient engagement	Per-member-per-month (PMPM) payments from all payers — Medicare, Medicaid, and commercial insurers — with a base payment plus a performance payment tied to quality outcomes	Transforms primary care practices from reactive, visit-based care to proactive, population-based care; coordinates care across providers; manages chronic conditions; screens for behavioral health and SDOH needs
Community Health Team (CHT)	Multi-disciplinary teams of social workers, nurses, community health workers, and behavioral health professionals embedded in or closely connected to primary care practices	Blueprint program funding through DVHA; supplemented by OneCare (now transitioning), Medicaid, and grant funding	Provides care coordination, SDOH navigation, behavioral health support, and connection to community services for patients with complex needs; bridges the medical-nonmedical divide
Health Information Technology	VITL-based health information exchange linking practices to the statewide clinical data registry and VHCURES all-payer claims database	VITL funding; Blueprint program funding for data analytics support	Enables population-level HEDIS measurement; provides practices with care gap reports and patient registries; supports quality improvement with data-driven insights

Figure 10.1 — Vermont Blueprint for Health: three integrated components. Sources: Blueprint for Health website; Vermont Blueprint Overview (GMCB, 2022); Blueprint Annual Report 2024.

The Evidence: What the Blueprint Has Produced

Vermont's Blueprint is one of the most extensively studied primary care transformation programs in the country. The evidence base spans more than a decade of operation and multiple independent evaluations.

The most rigorous evaluation — a six-year longitudinal study using VHCURES all-payer claims data published in Population Health Management — compared Blueprint participants to non-Blueprint primary care populations and found consistent advantages across expenditure, utilization, and quality outcomes. The critical financial finding: medical expenditures decreased by approximately \$5.8 million for every \$1 million spent on the Blueprint initiative. This is a 5.8:1 return on investment, driven primarily by lower hospitalization rates and reduced outpatient facility use — precisely the kinds of avoidable utilization that Oliver Wyman identified as the largest source of indirect savings from Vermont's transformation.

Medicare specifically found statistically significant healthcare savings for its members in Vermont's Blueprint program. The Blueprint's own annual ROI analyses, comparing the Blueprint period to the pre-Blueprint baseline and to non-participating practices, consistently showed a positive all-payer return on investment. The finding that Blueprint patients had better outcomes across payer types — including Medicaid beneficiaries, who typically face the most barriers to effective primary care — is particularly significant for Vermont's equity goals.

Blueprint ROI: medical expenditure reduction per \$1M invested	Vermont residents with a personal health care provider
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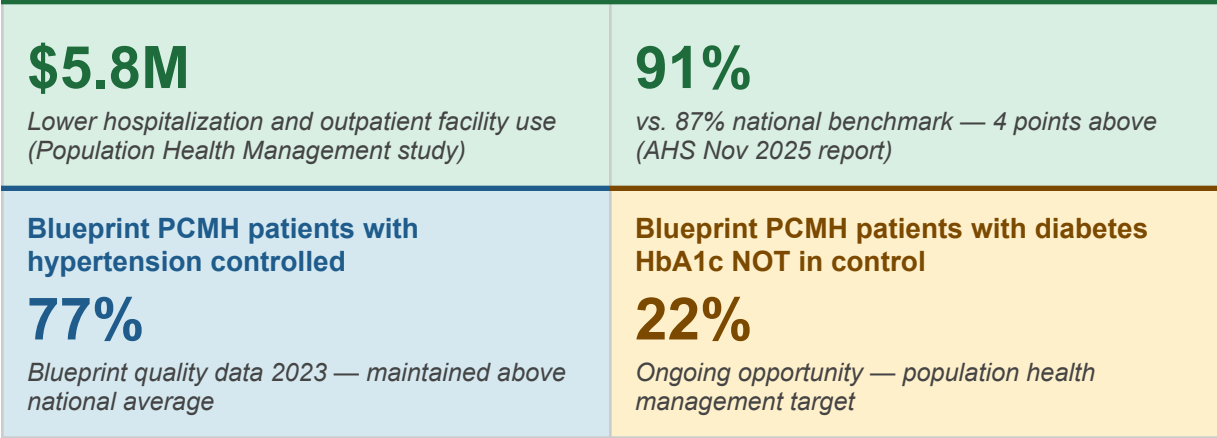


Figure 10.2 — Blueprint for Health clinical outcomes and ROI. Sources: Population Health Management journal; AHS November 2025 Transformation Report; Blueprint for Health 2024 data.

The Blueprint and AHEAD: A Critical Transition

Vermont's Blueprint program has operated since 2010 under a CMS Multi-Payer Advanced Primary Care Practice Demonstration that allowed Medicare to participate in PCMH and CHT payments. That agreement is scheduled to end with the conclusion of the All-Payer ACO Model. The AHEAD State Agreement resolves this transition risk: Primary Care AHEAD provides a voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models.

The transition from the current Medicare Blueprint payment to Primary Care AHEAD is operationally significant. Practices that currently receive Blueprint PMPM payments for Medicare patients will need to evaluate whether to participate in PC AHEAD, understand the new payment pathway options, and potentially redesign their care model to meet AHEAD's enhanced primary care requirements. AHS and DVHA have committed to supporting practices through this transition, recognizing that disrupting the Blueprint's Medicare payment stream would undermine one of Vermont's most effective clinical programs at precisely the moment transformation demands more of it.

Vermont's Blueprint enrollment target under AHEAD is ambitious: the program aims to substantially increase the percentage of Vermont's population — and particularly its Medicare population — attributed to Blueprint-participating practices. This expansion requires both new practice recruitment and support for existing practices to deepen their care management capabilities.

Vermont in Practice: The Blueprint Quality Measure Crosswalk

Vermont's primary care quality landscape is unusually complex because practices must simultaneously meet the quality requirements of multiple programs: Blueprint PCMH standards, AHEAD quality metrics, HEDIS measures used by commercial payers, and GMCB's hospital quality framework. Oliver Wyman documented the fragmentation this creates — multiple measurement systems with overlapping but not identical measures, creating administrative burden for practices trying to optimize across all of them.

AHS and Blueprint have developed a Quality Measure Crosswalk — an interactive tool that maps measures across programs, identifies overlaps, and helps practices understand which quality investments serve multiple accountability requirements simultaneously. This is operationally important: a primary care practice that installs a hypertension management protocol does not need to report separately to Blueprint, AHEAD, and commercial payers if those programs are aligned around the same clinical measure.

The crosswalk also reveals the gaps — places where programs require incompatible measures, creating impossible choices for practices with limited data infrastructure. These gaps are a specific target for the AHEAD implementation process and the Statewide Strategic Plan's quality alignment provisions.

Section 2: The Behavioral Health Crisis — Vermont's Most Urgent Clinical Challenge

Vermont's behavioral health data, documented in the November 2025 AHS transformation report, describes a system in crisis. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — and with 30-day follow-up after mental health ED visits reaching only 76%, Vermont's behavioral health system is failing patients at the point of highest acuity. The crisis presentation data is not just a quality indicator; it is evidence of system-level failure to provide adequate access to behavioral health services before patients reach the emergency department.

Oliver Wyman's community meetings documented the texture of this failure in qualitative terms: patients with limited access to mental health services; care that is discriminatory for people who don't speak English as a first language; an observed weight bias and fatphobia making it difficult for patients to get appropriate care. The quantitative and qualitative data together describe a behavioral health system that is overwhelmed, under-resourced, and inequitably distributed — concentrated near urban centers and inaccessible in the rural communities where need is often greatest.

The Three-Layer Behavioral Health Architecture Vermont Is Building

Vermont is developing a behavioral health system with three complementary layers that together can serve the full spectrum of mental health and substance use disorder needs — from universal screening in primary care to intensive community treatment to acute crisis response.

Layer 1	Mental Health Integration in Primary Care (CoCM and MHI) The Collaborative Care Model (CoCM) is the only integrated behavioral health model with designated Medicare billing codes and a 90+ randomized controlled trial evidence base. CoCM embeds a behavioral health care manager (BHCM) and consulting psychiatrist within a primary care practice, enabling screening, brief intervention, and treatment for low-to-moderate acuity mental health and SUD conditions without referral. Vermont's Mental Health Integration pilot (MHI) through the Blueprint embedded mental health clinicians and community health workers in primary care settings starting in 2023, with nearly 80% of administrative entities reporting increased CHT staffing. The 2025 evaluation found improved access and engagement but sustainability concerns when temporary funding ends — a direct argument for making MHI permanent through AHEAD Primary Care funding.
Layer 2	Certified Community-Based integrated Health Centers (CCBHCs) Vermont became an official CCBHC Demonstration State in 2024, receiving enhanced Medicaid funding to expand community behavioral health services. As of 2025, Vermont has two active CCBHC demonstration sites. Vermont's RHT Program application plans to certify five additional CCBHC entities by July 2026 — Health Care and Rehabilitation Services (Springfield), Howard Center (Burlington), Northwest Counseling and Support Services (St. Albans), Northeast Kingdom Human Services (Newport), and Washington County Mental Health Services (Barre). CCBHCs provide comprehensive behavioral health services including crisis intervention, substance use treatment, and care coordination — welcoming all patients regardless of age, diagnosis, or insurance status.
Layer 3	Crisis System and Acute Behavioral Health Infrastructure Vermont has developed mobile crisis response (operational since January 2024), centralized dispatch from the 988 Suicide and Crisis Lifeline, and new psychiatric residential treatment facilities for youth and forensic populations. Vermont's six 'Designated Hospitals' have inpatient psychiatric units; the Brattleboro Retreat serves as the state's primary psychiatric hospital. AHS has added hospital-level youth mental health beds, psychiatric residential treatment beds, and enhanced transport to the Brattleboro Retreat as part of its Act 167-mandated stabilization actions. The 30-day follow-up rate of 76% after mental health ED visits — while above national

averages for some measures — indicates that crisis presentations are not consistently connected to ongoing care.

The Collaborative Care Model: Clinical Standard of Practice for Primary Care Integration

The Collaborative Care Model merits extended treatment because it is the clinical standard of practice for behavioral health integration in primary care — not just a promising approach, but the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes (CPT codes 99492, 99493, 99494). Understanding CoCM's structure is important for any primary care or health system leader contemplating behavioral health integration.

CoCM component	Role	What they do	Vermont application
Primary Care Provider (PCP)	Treating and billing provider	Identifies patients with behavioral health needs through validated screening (PHQ-9, GAD-7); refers to BHCM; provides warm handoffs; bills CoCM codes monthly	Blueprint PCMH practices are the natural CoCM deployment sites — they already have the population management infrastructure and quality reporting that CoCM requires
Behavioral Health Care Manager (BHCM)	Day-to-day care coordinator	Maintains patient registry; provides brief interventions; monitors treatment response with validated rating scales; escalates to psychiatric consultant as needed; coordinates with community resources	MHI pilot found 80% of administrative entities increased CHT staffing — CHT staff are the natural BHCM pool; sustainability challenge when pilot funding ends
Psychiatric Consultant	Clinical supervision	Regular case review with BHCM; consultation on treatment recommendations; direct care for highest-acuity cases requiring psychiatric input; not the primary provider for most CoCM patients	Vermont's psychiatric shortage makes the CoCM model especially valuable — one psychiatrist can support an entire primary care practice's behavioral health population through consultation rather than direct care

Figure 10.3 — Collaborative Care Model components and Vermont application. Sources: SAMHSA CoCM program; CMS Behavioral Health Integration Services (January 2026); Vermont MHI Pilot Evaluation (2025).

The financial sustainability of CoCM in Vermont's environment depends on Medicare and Medicaid coverage of the billing codes, which Act 68 and AHEAD together support. Vermont Medicaid's coverage of CoCM codes is a prerequisite for practices serving high-Medicaid rural populations to implement the model — and DVHA's coordination with the Blueprint and AHEAD implementation is the mechanism for ensuring that coverage is in place. The AHEAD EAST Fund's explicit investment priority in mental health and substance use disorder services is the vehicle for funding the BHCM positions that primary care practices need to implement CoCM at scale.

"The MHI initiative allowed primary care teams to serve more patients and provide more consistent follow-up. Flexible and team-based care models helped patients engage in services, particularly those facing complex medical and social challenges."

— Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation, 2025

The SUD Continuum: Vermont's Hub and Spoke Model

Vermont's substance use disorder treatment system is organized around a Hub and Spoke model that has been nationally recognized as a model for rural SUD treatment. Hubs provide intensive addiction treatment services; Spokes — office-based opioid treatment (OBOT) practices — provide medication-assisted treatment in primary care settings. The Blueprint's practice finder maps active Spoke locations across Vermont's primary care network. AHS actions consistent with Act 167 have expanded this system: rate increases for residential SUD treatment, creation of 'hublets' in treatment deserts, support for co-occurring mental health and SUD treatment at hubs, contingency management for stimulant use disorder, and completion of gaps analyses to identify communities without adequate access.

The 30-day follow-up rate after ED visits for alcohol use — 68% in the November 2025 data — indicates that the care coordination infrastructure between crisis and community-based SUD treatment is not fully functional. Vermont is making specific investments to close this gap through the CCBHC expansion and the MHI permanent embedding of behavioral health in primary care, but the transition from crisis to community care remains the most operationally challenging step in the SUD care continuum.

Section 3: Primary Care Redesign — Vermont's Strategy for the Front Door

Oliver Wyman's prescription for Vermont's primary care system was both clear and counterintuitive: Vermont does not have a primary care physician shortage in the traditional sense. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is that primary care is not operating at the productivity and care model levels needed to serve Vermont's population. If primary care providers were supported to see three patients per hour, operating in team-based care models where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have sufficient primary care supply for well into the future.

This finding has profound implications for Vermont's primary care strategy. Rather than focusing exclusively on recruiting more physicians — an expensive, slow, and uncertain strategy — Vermont can meaningfully increase primary care capacity by transforming how existing primary care operates. The Blueprint's PCMH model is the vehicle for this transformation; AHEAD's enhanced primary care payments are the financial fuel.

The Vermont Comprehensive Primary Health Care Initiative

Act 68 established the Vermont Steering Committee for Comprehensive Primary Health Care — a body whose recommendations must be incorporated into the Health Care Delivery Advisory Committee's work and AHS's Statewide Strategic Plan. This committee represents a policy commitment to treating primary care not as one service line among many but as the foundational layer of Vermont's reformed delivery system — the front door through which most Vermonters access the healthcare system and the setting where most disease management, behavioral health integration, and SDOH screening should occur.

Vermont's comprehensive primary health care vision, as embedded in Act 68 and the AHEAD State Agreement, has five operational components:

Component	What it means in practice	Vermont investment vehicle
Team-based care	Every PCMH practice operates with a full care team — physician or APRN, medical assistant, care coordinator, behavioral health care manager, and CHT connection — with each team member working at the top of their licensure. The physician's time is reserved for the clinical decisions only a physician can make.	Blueprint PCMH payments; AHEAD enhanced primary care payments; RHT Program workforce investment in APRN training and scope expansion
Population health management	Every PCMH practice maintains a panel registry, tracks care gaps proactively, and reaches out to patients who need preventive care, chronic disease management, or behavioral health support before they present in crisis.	Blueprint data infrastructure; VHCURES linkage; GMCB analytics vendor procurement
SDOH screening and navigation	Universal screening for health-related social needs (housing, food security, transportation, financial insecurity) at every primary care visit, with warm handoffs to CHT staff for navigation and community resource connection.	Blueprint HRSN screening expansion; AHEAD EAST Fund SDOH investments; RHT Program community health worker deployment
Behavioral health integration	CoCM or equivalent behavioral health integration at every PCMH practice, providing screening, brief intervention, and care management for mental health and SUD conditions without requiring a separate specialty referral.	MHI pilot expansion through AHEAD and EAST Fund; CCBHC partnerships with primary care
Care coordination for complex patients	Intensive care management for Vermont's highest-cost, highest-complexity patients	CHT staff; dual eligible care planning; AHEAD population

	— the elderly with multiple chronic conditions, dual-eligible Medicare/Medicaid beneficiaries, individuals with serious mental illness — linking primary care to specialist care, post-acute care, and community support.	health requirements; Blueprint Chronic Care Initiative
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Figure 10.4 — Vermont comprehensive primary health care: five operational components and investment vehicles. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Plans 2025.

Primary Care's Role in Reducing Hospital Utilization

The connection between primary care investment and hospital utilization reduction is one of the best-established relationships in health services research, and Vermont's own Blueprint data confirm it. The 2013 evaluation finding — lower hospitalization rates and reduced outpatient facility use for Blueprint participants — is the mechanism through which primary care investment produces hospital savings under a global budget.

Vermont's November 2025 data shows 32.3% of all ED visits are potentially avoidable — meaning they represent primary care access failures, not genuine emergency needs. At Vermont's scale (approximately 14 hospitals, roughly 200,000+ ED visits annually across the system), reducing potentially avoidable ED visits from 32.3% to a benchmark level of 25% would eliminate approximately 14,600 ED visits per year. At average ED visit costs in the \$800-1,200 range, this reduction represents \$12-18 million in annual savings — and that is before accounting for the downstream hospital admissions from avoidable ED visits that could have been prevented by earlier primary care intervention.

This is why Act 68's explicit goal of increased investment in primary care is not a soft preference — it is a financial necessity. Under global budgets, every avoidable hospitalization or ED visit is a direct cost without offsetting revenue. Hospitals operating under global budgets have a powerful financial incentive to invest in the primary care infrastructure that keeps their attributed population out of the hospital. Vermont's AHEAD EAST Fund, with its explicit investment priority in primary care, is designed to accelerate this investment.

Section 4: Vermont's Clinical Quality Architecture — Measuring What Matters

Clinical quality measurement in Vermont operates through multiple overlapping frameworks that were not designed to be coherent with each other. HEDIS measures used by commercial payers, Blueprint quality measures for PCMH payments, GMCB hospital quality reporting, AHEAD accountability metrics, and Act 167/Act 68 outcome measures all exist simultaneously, creating a measurement burden that Oliver Wyman specifically cited as a driver of administrative complexity and provider frustration.

The Statewide Strategic Plan that AHS must deliver by December 2028 presents the opportunity — and creates the obligation — to align Vermont's clinical quality measurement framework around a coherent set of indicators that serve multiple accountability purposes simultaneously. The following framework represents the clinical quality architecture that Vermont's reform requires:

Domain	Priority measures	Why this measure	Current VT status
Preventive and chronic care	Hypertension control rate; diabetes HbA1c management; cervical cancer screening; colorectal cancer screening; childhood immunization	Chronic disease management is the highest-volume primary care activity; these measures predict long-term utilization and cost	Hypertension 77% controlled; diabetes 22% not controlled. Both reflect Blueprint PCMH performance; non-Blueprint practices likely worse
Behavioral health access and quality	30-day follow-up after MH ED visit; 30-day follow-up after SUD ED visit; depression screening rate; SUD treatment initiation and engagement	ED visits for MH/SUD are the clearest evidence of access failure; follow-up rates measure whether crisis intervention connects to ongoing care	76% 30-day follow-up after MH ED visit; 68% after SUD ED visit — both indicate coordination gaps
Primary care access	% practices accepting new Medicaid patients; wait time for new patient appointment; % population with attributed primary care provider	Access to primary care is the front door to the system; Medicaid acceptance is the equity indicator	59% of primary care practices accepting new Medicaid patients — significant equity gap; 91% of Vermonters have a personal health provider
Hospital quality and efficiency	30-day all-cause readmission rate; potentially avoidable ED visit rate; operating profit per adjusted discharge; admin cost per adjusted discharge	Hospital metrics measure both clinical quality and financial sustainability	Readmission: 14.8% (within range). Avoidable ED: 32.3% (major opportunity). Admin cost: 91% above benchmark
Equity and SDOH	Health outcome variation by county, income, race/ethnicity; insurance coverage rate by geography; SDOH screening completion rate; community resource referral completion	Equity measures are required by Act 68 and Act 167; geographic variation within Vermont is large and not adequately measured	Northeast Kingdom uninsured 6-8% vs. 3% statewide; rural mental health access gaps; SDOH screening completeness not consistently tracked

Figure 10.5 — Vermont clinical quality measurement framework: priority measures, rationale, and current status. Sources: AHS Transformation Report (November 2025); Blueprint for Health data; Oliver Wyman Act 167 Report.

The HEDIS Improvement Methodology — Vermont Context

The HTR HEDIS improvement methodology — five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices and hospitals participating in Blueprint and AHEAD. The Vermont-specific context adds three considerations:

- All-payer measurement is both an advantage and a challenge. Vermont's VHCURES all-payer claims database, linked to the Blueprint clinical data registry, enables HEDIS measurement across the full population — not just the commercial population a health plan would report on. This means Vermont can identify disparities that would be invisible in single-payer reporting. The challenge is that the database requires VITL connectivity that remains voluntary and incomplete, particularly among community-based providers.
- Blueprint field staff are the quality improvement infrastructure. Unlike states where practices must manage their own quality improvement in isolation, Vermont's Blueprint field staff — one Quality Improvement Facilitator per HSA region — provide ongoing support, data analysis, and evidence-based practice coaching. This infrastructure is a competitive advantage for Vermont's transformation; it means quality improvement capacity is not limited by the resources of individual practices.
- Performance payment alignment creates a direct financial incentive for quality improvement. Blueprint's PMPM payments include a performance payment component tied to quality outcomes and utilization measures. AHEAD's enhanced primary care payments include quality-based adjustments. This means Vermont primary care practices have a direct financial incentive for HEDIS improvement — not just a regulatory compliance requirement.

Section 5: Care for Vermont's Aging Population — The Clinical Imperative

Vermont's demographic trajectory — 65+ population growing 57% by 2040, exceeding 30% of total population — creates a clinical imperative that is simultaneously a financial imperative. The aging population drives demand for the most expensive and most complex services: cancer treatment, cardiac surgery, dementia care, frailty-related hospitalization, long-term care, end-of-life care. Designing the clinical system to serve this population well — which means keeping elderly Vermonters healthy, connected to their communities, and out of hospitals and nursing homes to the extent clinically appropriate — is both the right thing to do and the financial requirement of the global budget model.

The PACE Model and Vermont's Long-Term Care Gap

The Program of All-Inclusive Care for the Elderly (PACE) integrates medical, social, and long-term care services for frail elderly patients who would otherwise require nursing home placement, enabling them to remain in their communities. PACE is one of the most effective and cost-efficient models for managing the population segment that drives the highest healthcare costs in any state system. Vermont has no PACE programs. This is a significant gap that Vermont's transformation planning must address.

Oliver Wyman's community meetings documented the dimensions of Vermont's long-term care capacity crisis: lack of staffed social care facilities leads to hospital crowding when patients cannot be discharged; nursing home occupancy is high and new capacity is constrained; home and community-based services (HCBS) — the preferred alternative to institutional care — are limited by workforce shortages. Vermont's RHT Program application specifically addresses long-term care gaps: funding

for dialysis units in nursing homes so dually eligible residents can receive long-term care without institutional transfer, and a second nursing home ventilator unit to double capacity for high-acuity nursing home care.

The clinical model for Vermont's aging population requires three integrated components: strong primary care for chronic disease management and prevention; robust HCBS and PACE to support aging in place; and smooth hospital-to-community transitions when hospitalization is unavoidable. The first component is the Blueprint's domain. The second requires the SDOH investment in housing and transportation that Oliver Wyman made Imperative 1. The third requires the care coordination infrastructure that the CHT model provides.

Dementia Care: The Coming Clinical Challenge

Dementia affects an estimated 14 million Americans nationally by 2060, and Vermont's older demographic profile means its per-capita rate of dementia will reach the national projection earlier. Vermont currently lacks dementia-specialized primary care at scale, adequate memory care capacity in its nursing home sector, and family caregiver support infrastructure proportionate to the approaching need. Oliver Wyman's COE framework designates memory care as a COE specialty — recognizing that dementia care at adequate quality requires specialized expertise and cannot be replicated at every community hospital.

The clinical pillar implications are clear: Vermont's transformation plan must include explicit investment in dementia care infrastructure — the training of primary care providers in cognitive assessment and early dementia management, the development of memory care COE capacity at designated facilities, the expansion of adult day programs and respite care for family caregivers, and the integration of dementia care into the community-based care model that Vermont's global budget environment requires.

Section 6: Clinical Pillar Implications for AHS's Statewide Strategic Plan

The Statewide Health Care Delivery Strategic Plan that AHS must deliver by December 2028 must address the clinical pillar explicitly. Based on Vermont's current clinical landscape and the transformation agenda established by Acts 167 and 68, the plan should contain five clinical design commitments:

Commitment	What it requires	Timeline	Primary vehicle
Universal Blueprint PCMH coverage	Every primary care practice in Vermont participates in Blueprint PCMH recognition and receives all-payer PMPM payments; no Vermont resident is more than 30 minutes from a PCMH practice	2025-2028	Blueprint expansion; AHEAD PC primary care recruitment; RHT workforce investment
Behavioral health integration at every PCMH	Every PCMH practice implements CoCM or equivalent behavioral health integration, with BHCM staff and psychiatric consultation available to all attributed patients	2025-2028	MHI permanent funding through AHEAD EAST Fund; CCBHC partnerships; BHCM workforce training through RHT Program
CCBHC network covering all HSAs	Certified Community-Based integrated Health Centers established in every Hospital Service Area, providing comprehensive community behavioral health services	2026 (5 new CCBHCs) through 2028	CCBHC Demonstration State enhanced Medicaid; RHT Program funding; Act 68 grants

	including crisis response, SUD treatment, and care coordination		
PACE program development	At least one PACE program operational in Vermont, serving the highest-acuity elderly population that would otherwise require nursing home placement; feasibility assessment for additional programs	2026-2028 (feasibility and launch)	AHEAD EAST Fund; RHT long-term care infrastructure investment; Medicaid PACE program rules
Dementia care infrastructure	Memory care COE designation at 3-5 facilities; primary care dementia assessment protocols embedded in all PCMH practices; family caregiver support programs in all HSAs	2026-2030	RHT Program; COE designation process; Blueprint quality measures expansion

Figure 10.6 — Clinical pillar commitments for Vermont's Statewide Health Care Delivery Strategic Plan.
Sources: Act 68 of 2025; Oliver Wyman Act 167 Report; AHS Transformation Plans; Vermont RHT Program Application.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Blueprint for Health	Vermont's all-payer primary care transformation program, operating since 2006. Provides per-member-per-month payments to NCQA-recognized Patient-Centered Medical Home practices and funds Community Health Teams to provide wraparound care coordination services. The foundational layer of Vermont's clinical pillar.
Patient-Centered Medical Home (PCMH)	A primary care practice recognized by NCQA as meeting advanced standards for care coordination, population management, quality improvement, and patient engagement. The organizational model for Vermont's primary care system under Blueprint and AHEAD.
Community Health Team (CHT)	Multi-disciplinary team of social workers, nurses, community health workers, and behavioral health professionals supporting PCMH practices with care coordination, SDOH navigation, and connection to community services.
Collaborative Care Model (CoCM)	The integrated behavioral health care model with the strongest evidence base — 90+ RCTs — embedding a behavioral health care manager and psychiatric consultant in primary care to address mental health and SUD needs without requiring specialty referral. The only integrated care model with designated Medicare billing codes.
Certified Community-Based Integrated Health Center (CCBHC)	Vermont's term for what CMS calls Certified Community Behavioral Health Clinics. Comprehensive community-based behavioral health facilities receiving enhanced Medicaid funding through the federal CCBHC Demonstration program.
Mental Health Integration (MHI)	Vermont Blueprint's pilot program embedding mental health clinicians and community health workers in primary care settings; evaluated in 2025 as highly effective but facing sustainability challenges when pilot funding ends.

PACE (Program of All-Inclusive Care for the Elderly)	An integrated medical, social, and long-term care model enabling frail elderly individuals to remain in their communities rather than entering nursing homes. Vermont currently has no PACE programs — a significant gap given its aging demographics.
Hub and Spoke	Vermont's SUD treatment model: Hubs provide intensive addiction treatment; Spokes are office-based opioid treatment practices in primary care settings providing medication-assisted treatment.
HRSN (Health-Related Social Needs)	The social needs that affect health outcomes — housing, food security, transportation, financial insecurity, social isolation — now universally screened for in Vermont's Blueprint primary care practices.

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program website (mentalhealth.vermont.gov); Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation (2025); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services Health Care System Transformation Report (November 2025); Act 68 of 2025; Vermont AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); Population Health Management (Blueprint ROI study, 2016); SAMHSA CoCM Program; CMS Behavioral Health Integration Services (January 2026).